



Acute Response Mental Health Emergencies/Crises

Policy code	SMHS:17
Original endorsement date	01 September 2003
Date of current endorsement	02 February 2024
Functional subgroup	Mental Health
Summary	Mental health services will endeavour to provide appropriate acute responses based on consumer needs.
Policy owner	Executive Director Clinical Service Planning and Population Health, Kate Gatti.
Contact	Health Service Planner, Anureet Johl, 6152 1169
Review date	02 February 2027
NSQHS Standards	Standard 1: Clinical Governance Standard 6 Communicating for safety Standard 8: Recognising and Responding to Acute Deterioration
Chief Psychiatrists Standards for Clinical Care Status	Assessment Risk assessment and management Active

1. Background

1.1. About this document

Mental health services utilise the triage process to determine the urgency of response (UoR) required for a consumer and to clearly indicate which service is required to act on the UoR.

1.2. Key definitions

The Triage to Discharge Mental Health Framework for State-wide Standardised Clinical Documentation includes the Crisis Triage Rating Scale (CTRS) (DoH NSW, 2020) which is a brief rating scale developed as a guideline for clinicians to screen emergency psychiatric consumers rapidly to determine the required level of response. The scale evaluates the consumer based on three factors; dangerousness, support system and ability to cooperate. The following minimum action / interventions should assist the triage clinician respond to consumer / referrer needs, as clinically indicated and discussion with MDT:

Category	Definition
A - Extreme urgency	Immediate response/intervention due to clear and imminent serious risk of harm to self or others. Very high unpredictability disorganisation, acute disturbance. May include police / ambulance notification.
B - High urgency	(Contact within 2 hours, where possible) Due to serious risk to self or others. High unpredictability, disorganisation, acute disturbance.
C - Medium urgency	(Contact within 12 hours, where possible) Due to harm, crisis, distress benefit of early intervention.
D – Low urgency	(Contact within 48 hours, where possible) Due to client / carer distress, early symptoms of psychosis, person known to the service requiring priority review.
E – Non-Urgent	(See within 2 weeks) A routine referral is for a non-urgent assessment which will be managed at the next intake meeting.

2. Policy Statement

All individuals, when experiencing a mental health crisis or deemed to be in a mental health emergency situation, will be provided with the appropriate contact needs and assessment based on available information. Staff should endeavour to collate as much information as is required to determine an appropriate response and record as a Triage contact within PSOLIS. A Risk Assessment and Management Plan (RAMP) will be completed for patients experiencing a mental health emergency. “Based on the information that has been obtained” with this statement included in the RAMP. The information gathered can be from multiple sources such as patient / family friends/ GP, referrer, previous records and PSOLIS.

All MH service actions / responses will be in accordance with the Mental Health Triage Rating Scale taking in to account the service’s ability at that time to respond. Assistance should be gained

from other services – mental health, police and ambulance as deemed appropriate in order to reduce risk.

3. Roles and Responsibilities

3.1. Executive

Each SMHS site or service executive will ensure managers and supervisors are aware of the requirement to implement guidelines and safe work procedures relating to Acute response mental health emergencies/crises.

3.2. Managers/Supervisors

Managers and supervisors should:

- Undertake risk assessments of tasks involving acute response mental health emergencies/crises
- Ensure the development of relevant protocols and procedures
- Ensure staff are trained in relevant safe work procedures
- Where available and appropriate, ensure appropriate training and education is provided to staff in relation to acute response mental health emergencies/crises

3.3. Staff

Staff should:

- Comply with mandatory training requirements and proactively seek further training as required
- Ask for assistance if required and involve relevant medical staff and MDT in the decision-making process regarding acute response mental health emergencies/crises.
- Comply with safe work procedures

4. Category A – Extreme urgency

4.1. Children and adolescents

Children and adolescents assessed as Category A will be referred using the identified procedure to Ambulance and / or Police Service:

- Urgent specialist advice can be provided by contacting Crisis Connect.
- Children and adolescents under sixteen years of age are to be referred to their local Emergency Department via Ambulance or Police Services.
- Adolescents between the ages of sixteen and eighteen years are to be referred to the nearest emergency department to their location; via the Ambulance or Police services.
- Children and adolescents who require protective arrangements and/or have been the subject of physical, sexual and/or emotional abuse are to be referred directly to the Department of Children's and Family Services, Perth Children's Hospital Child Protection Unit and/or the Police Child Protection Squad.

4.2. Adults and older adults

Adults and older adults assessed as Category A are to be referred to Ambulance or Police services in accordance with the Requesting Road-Based Transport for Mental Health Patients Subject to Transport Orders Policy.

5. Category B, C, D – High, Medium and Low Urgency

5.1. Children and adolescents

Children and adolescents assessed as UoR Category B, C, & D will be referred appropriately: Where the degree of risk of harm to self or others is high, the receiving clinician is to take immediate action to involve the Child and Adolescent Mental Health Service (CAMHS). CAMHS is to be informed about the urgent referral and a plan for follow-up documented. CAMHS will assist with the coordination of other government and non-government mental health services.

5.2. Adults and older adults

Adults and older adults assessed as Category B, C or D will be referred using the identified procedure.

- During office hours, ATT will discuss acute referrals with the referring person in order to coordinate the most appropriate action with the assistance of members of the Community Treatment Team (CTT) and triage officer, where required.
- The Triage Officer will process all new referrals and re-referrals of past clients. This involves:
 - Receiving the initial information
 - Completing a Triage form including the assessment of UoR required
 - Checking if the person has had previous contact with the service
 - Obtaining collateral information, and
 - Documenting all pertinent information to be taken to the next Intake Meeting.
- If ATT/triage officer are unable to respond within the recommended timeframe patient / family/ referrer will be informed, and arrangements made for safe transfer to the Emergency Department.
- PS not just ATT / CTT/ triage that cannot respond, if CTT patient cannot be responded to same applies – we have all had a patient ring at 4.29pm on a Friday who is suicidal

6. After hours emergency

- ATT/Triage Officer will manage mental health emergencies that self-present until 22:00hrs.
- Staff aware of a consumer whom they consider to be of clear and imminent risk of a psychiatric emergency after business hours should alert the ATT.
- If that patient is at imminent risk of harm staff will contact the Ambulance or Police Services.
- Acute presentations will be triaged at the nearest Emergency Department.

After 2000hrs:

- Acute response / crisis calls are triaged by the Mental Health Emergency Response Line (MHERL).

7. Compliance monitoring and legislative penalties

7.1. Compliance monitoring

Each SMHS site MH Service Director/Co-Director is to ensure compliance with this policy.

Compliance monitoring will include but is not limited to the following:

- Number of referrals per CTRS Triage severity
- Number of referrals seen within Our timeframe

7.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

7.3. Legislation

- Mental Health Act 2014

7.4. Other

- Mental Health Triage Policy, Department of Health NSW, 2012
- Chief Psychiatrists Standards for Clinical Care
- ['Triage to Discharge' Mental Health Framework for State-wide Standardised Clinical Documentation](#)
- Community Mental Health Status Assessments Role of Mental Health Clinicians - MP 0099/18
- Safety Planning for Mental Health Consumers Policy
- Requesting Road-Based Transport for Mental Health Patients Subject to Transport Orders Policy
- MP 0165/21 Use of Physical and/or Mechanical Restraint during Road-based Transportation of Mental Health Patients Policy
- Emergency mental health contact details
- National Consensus Statement: Essential elements for recognising and responding to deterioration in a person's mental state
- National Safety and Quality Health Service Standards Standard 8 - Recognising and Responding To Acute Deterioration
- National Standards for Mental Health Services 2010

7.5. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP 3	05/02/2024	02/02/2024	02/02/2027	2027	

7.6. Approval

Approval by	Executive Director Clinical Service Planning and Population Health, Kate Gatti
Approval date	02 February 2024

This document can be made available in alternative formats on request.

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